



A SOOTHEMADE BOOK

30 Scripts for the Postpartum Visitor

*What to say when you don't have the energy
to find your own words.*

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the parent at the door, on the third day, in the fourth trimester.

You were always allowed to say no.

A note from Maya

The doorbell rang for the fourth time on the third day home from the hospital. I had been awake for thirty-eight hours. The baby had been latched for the previous ninety minutes. The doorbell ringer was my mother-in-law, holding a tray of cookies, and she expected to be let in.

I cried. I let her in. The visit was four hours long.

I made the printable version of this pack so the next time someone did that to me, I would have words. Then I realized I needed it in a form I could read at 3am on my phone, when the texts were coming in and my brain had stopped giving me sentences. So this book exists.

It is not because I am bad at boundaries. It is because postpartum exhaustion eats words first. The brain that used to know how to say "no thanks" is gone for a stretch. You need pre-made language, and you need it where you can find it.

So here are the words. Pre-made. Use them, modify them, hand them to your partner. None of these scripts require you to justify yourself. Saying no is a complete sentence, but if you want a softer landing, that is here too.

You do not owe anyone access to your body, your baby, your house, or your decisions. Not your in-laws. Not your best friend. Not your mother. The fact that they love you does not entitle them to your fourth trimester.

The book is short on purpose. You can read it in one nursing session. Save it to your phone. Send the relevant chapter to your partner. Use what you need.

With care, Maya

Chapter 1: Why "No" Is Hard Postpartum

There is a specific kind of word-loss that happens after birth.

It is not the dreamy first-week haze, though that is part of it. It is not "baby brain," that condescending phrase people use to mean "your priorities have changed." It is something more clinical and more brutal. The hormonal cliff, the sleep loss, the physical recovery from one of the most intense events a human body does, all of those things eat your executive function for a stretch. The you who used to manage a calendar, hold a complicated conversation, write a polite-but-firm email, that person is offline.

She comes back. Not on the timeline anyone promises. Slowly. In pieces. By six weeks you can usually finish a sentence again. By twelve weeks you can usually hold an argument. By six months you can usually return to the version of yourself who could say "no thanks, that does not work for me" in real time.

In the meantime, the world keeps showing up at the door.

The mother-in-law arrives with cookies. The cousin posts the hospital photo without asking. The aunt sends the third "when can I meet the baby" text in two days. The friend says "I will just drop by for a minute" and stays for two hours. The grandparent corners you in the kitchen and asks why you are not doing the feeding the way they did.

Each of these moments requires you to do a thing your brain has temporarily lost the equipment for. To say no. To stand firm against pressure. To not get talked out of your own boundaries.

To not cry, again, in front of a person who loves you and is also overwhelming you.

The book is for those moments.

The scripts are not harsh. They are not for an enemy. They are for the people who love you and are also too much. The voice is warm-direct, the way a slightly-older friend would coach you through this if she were sitting next to you on the couch at 3am.

A small note before we start. Postpartum is the season where "no" is hardest and also the season where it matters most. Every yes you give to a visitor you did not want is a yes you took from your own recovery, your own sleep, your own quiet bonding time with the baby. The math is not metaphorical. The visit costs you something measurable. You are allowed to say no to that visit.

You are allowed to say no.

You are still you. You are exactly you. You are just you with new information about what you need.



Chapter 2: Hospital Visitors

The hospital is the first wave.

For some people, the wave is small: a partner, maybe a parent, a quiet room. For others, the wave is enormous: cousins arriving from three states away with the assumption that they will be let in to hold the baby within hours of delivery. The hospital staff cannot stop most of this. The boundary has to come from you, or from your partner on your behalf.

What helps: setting the expectation before the birth, so the conversation is not happening in real time while you are in the recovery room. The five scripts in this chapter are pre-emptive (Script 1.1), in-the-moment (Scripts 1.2 to 1.4), and physical (Script 1.5, a sign for the door).

You do not need a medical reason. You do not need an excuse. You can say "we are not having visitors at the hospital" and that is a complete sentence. Some people will respect it. Some will not. The scripts below are calibrated for both kinds.

SCRIPT 1.1, THE PRE-EMPTIVE ANNOUNCEMENT (SEND BEFORE BIRTH)

Hi everyone, quick note as we head into the home stretch. We're not having any visitors at the hospital. We'll share photos as soon as we can, and we'll let you know when we're ready for people to come meet [baby] at home. Thanks for understanding, it means a lot. ♥

SCRIPT 1.2, WHEN SOMEONE "JUST STOPS BY" ANYWAY

Hey, thanks for thinking of us. We're not seeing visitors right now. I'll text you when we're ready. Appreciate it.

SCRIPT 1.3, THE "I HAD NO IDEA" PERSON

Oh, I'm so sorry, I should have been clearer. We're not having any visitors this trip. Let me text you when we're back home and settled, and we'll set up a real visit then.

SCRIPT 1.4, FOR YOUR PARTNER TO USE AT THE DOOR

Hey [name], thanks for coming by. [Birthing parent] is resting and can't have visitors right now. I'll have them text you when they're up for it.

SCRIPT 1.5, FOR THE NURSES (A SIGN FOR THE DOOR)

Please print and tape to the hospital room door.

Welcome, we're so glad to meet our new baby and we appreciate your interest in our family. We are not accepting visitors at this time. We will share updates and photos when we can. If you have a delivery or a question, please leave a note with the front desk. Thank you.

Chapter 3: The First Week at Home

The hospital was the first wave. Home is the bigger one.

The bigger one because people who would not have shown up at the hospital absolutely will show up at your house. The neighbor with a casserole. The friend with a stuffed animal who "just wants a quick peek." The relative who texts "we are in the area" and means "we are pulling into your driveway." Each one well-meaning. Each one with a different idea of what a "short visit" means than you do.

The math of the first week: a thirty-minute visit costs you roughly half a day of recovery. The baby is not the energy drain. The performance of hosting is. Getting dressed, presenting the house, holding a conversation while leaking through your shirt, smiling on no sleep, that work compounds. By the time the visitor leaves, you have lost a nap and your last good outfit.

So the scripts in this chapter are for cushioning the doorbell. The "I'll drop off food and just say hi" person (no, leave it on the porch). The "I won't stay long" person (a real visit costs you a recovery day right now, see you in three weeks). The mother-in-law who has surprised you (I love that you wanted to surprise us, I cannot host you right now).

You can be warm and firm at the same time. The scripts model both.

SCRIPT 2.1, THE "CAN I COME HOLD THE BABY?" TEXT

Thanks for thinking of us! We're not ready for visitors yet, the first couple weeks we're keeping it small. I'll let you know when we're ready. ♥

SCRIPT 2.2, THE "I'LL BRING FOOD AND JUST DROP IT OFF!" PERSON

That is so kind of you. If you really want to help, could you leave it on the porch? We're keeping the house quiet right now. We'll text you a thank-you when we surface.

SCRIPT 2.3, THE "BUT I WON'T STAY LONG" PERSON

I know you mean well, and I appreciate it. For us "short visits" still cost us a recovery day right now. Can we plan something properly in [X weeks] instead?

SCRIPT 2.4, THE MOTHER-IN-LAW WHO ARRIVED AT THE DOOR UNINVITED

[MIL name], I love that you wanted to surprise us. I'm not in a state to host right now. Can I call you in a few hours and we'll plan a real visit?

SCRIPT 2.5, WHEN THE VISITOR WANTS TO HOLD THE BABY AND THEY'RE SICK

Hey, I see you've got a bit of a cough/cold. We're being really careful with [baby]'s exposure right now. Can we reschedule for when you're 100%?

Chapter 4: Unsolicited Advice

Advice in postpartum lands differently than advice at other times of life.

At a baby shower, "have you tried this swaddle" is a friendly suggestion. At three in the morning on day four, with a baby who has been crying for forty-five minutes and a stitched-up body and a partner asleep on the couch, the same sentence is an attack. It is not because the person meant it as one. It is because the receiving system is depleted. Every input is amplified.

Most of the people giving you advice mean well. Some genuinely think they are helping. Some are nostalgic for their own postpartum and reliving it through you. A few are insecure about how they did it and are looking for confirmation that you will do the same. None of those motivations is yours to manage. Your job is to deflect the input efficiently and keep moving.

The eight scripts in this chapter are short on purpose. The longer your response, the more rope you give the advice-giver to argue. "Thanks, we are working with our pediatrician" is a complete sentence. "Oh, that is interesting, well in our case, the doctor said, and our family situation, and we are trying to balance" is six sentences they can pull on.

A small rule: never JADE. Justify, Argue, Defend, Explain. Pick one warm-firm sentence. End there.

SCRIPT 3.1, THE "YOU SHOULD REALLY BE DOING X" PERSON

Thanks, we're following our pediatrician's recommendations on that. Appreciate you looking out for us.

SCRIPT 3.2, THE "IN MY DAY WE..." PERSON

I love hearing about how you did it. We're trying things our way and figuring it out as we go.

SCRIPT 3.3, THE BREASTFEEDING-OPINION PERSON

Feeding is going well for us. I'm not really looking for input on this one.

SCRIPT 3.4, THE SLEEP-TRAINING-OPINION PERSON

We're working it out with our pediatrician. Thanks for caring.

SCRIPT 3.5, THE BODY-COMMENT PERSON ("YOU LOOK SO [X]!")

"Thanks!", and walk away. No second sentence required.

SCRIPT 3.6, THE "IS THE BABY GETTING ENOUGH?" PERSON

The baby is doing great. Thanks for asking.

SCRIPT 3.7, THE "YOU'RE GOING TO SPOIL THAT BABY" PERSON

You can't spoil a baby this young. The research is pretty clear. But thanks!

SCRIPT 3.8, THE "WHEN ARE YOU HAVING THE NEXT ONE?" PERSON

Oh that's a long way off. We're focused on this one right now.

Chapter 5: Visitor-Pressure Family

This is the hardest chapter.

The hardest because the pressure is not from strangers. It is from the people who love you and refuse to hear no. The grandmother who is "heartbroken" she has not met the baby. The aunt who drove four hours and "deserves" to hold them. The relative who shows up at your door uninvited and acts surprised when you are not delighted.

The scripts in this chapter are calibrated for love-with-pressure. They acknowledge the love. They do not capitulate to the pressure. That balance is the work.

A few principles for the hard ones:

- You can say "I love you and I cannot host you right now" in the same sentence. The love and the no live together.
- "I am doing what I think is right for my family" is a complete answer. You do not have to defend the strategy.
- Comparisons (so-and-so got to see the baby first, but I am family, but I waited longer) are not arguments. They are bids for guilt. You do not have to engage with them.
- The "you will regret this" line is the loudest tell that the speaker is dysregulated. Their fear of being shut out is not your problem to solve. The version of you in ten years will not regret protecting the baby in the first weeks. They will regret the visits they did not say no to.

If the conversation gets too hard, Script 4.5 is the one to memorize. "I love you, and I cannot host you right now. I need you to go. I will call you tomorrow."

That is allowed.

SCRIPT 4.1, THE "WE'RE DRIVING FOUR HOURS, WE HAVE TO SEE THE BABY" RELATIVE

I love that you want to be part of this. We are not seeing anyone right now for [X] weeks. I know that's hard, I'm sorry. We will set up a real visit when we're ready.

SCRIPT 4.2, THE "YOUR MOTHER IS HEARTBROKEN SHE HASN'T MET THE BABY" GUILT-TRIPPER

I know. I love her. We're doing it on our timeline. She'll meet [baby] when we're ready.

SCRIPT 4.3, THE "BUT EVERYONE ELSE GOT TO SEE THE BABY" COMPARISON

What works for one person doesn't work for everyone. We're doing this our way.

SCRIPT 4.4, THE "I WAITED THIS LONG, I DESERVE TO HOLD THEM" PERSON

I appreciate your patience. We're not at the holding-the-baby stage yet for [reason, illness, RSV season, my recovery, whatever]. We'll get there.

SCRIPT 4.5, WHEN A FAMILY MEMBER JUST SHOWS UP

[Name], I love you, and I cannot host you right now. I need you to go. I'll call you tomorrow.

SCRIPT 4.6, THE "YOU'LL REGRET KEEPING THE BABY FROM FAMILY" GUILT

I'm doing what I think is right for my family right now. I won't regret protecting [baby] in the first weeks.

Chapter 6: Photos and Social Media

Photos of a baby are different from photos of anything else.

Once a photo is online, it cannot be unposted. The aunt who posts the hospital photo to her two thousand Facebook friends has made a decision for you that you cannot un-make. The relative who sends the photo of your baby to their cousins' group text has multiplied the audience in a way you did not consent to.

For some families, this is a small thing. For some, it is not. There are real reasons people want their child's image not on social platforms: privacy, custody situations, religious or cultural reasons, simply the right to decide. None of the reasons need defending.

The three scripts in this chapter cover the most common scenarios. The "can I share this" relative (set the rule in advance). The "I already posted" relative (ask them to take it down, warmly, the first time). The grandparent who wants a "professional shoot ASAP" (we will do photos when we are ready).

A small principle: a polite ask the first time, a firm ask the second time. If a relative posts again after being asked not to, you have learned something about how they handle your requests. That information is useful.

SCRIPT 5.1, THE "CAN I SHARE THIS PICTURE?" RELATIVE

Thanks for asking! We're keeping [baby] off social media for now. Please keep our photos private.

SCRIPT 5.2, WHEN A RELATIVE ALREADY POSTED

Hi, I see you posted that photo of [baby]. We've decided to keep [their] photos off social media. Can you take it down? I really appreciate it.

SCRIPT 5.3, THE GRANDPARENT WHO WANTS A "PROFESSIONAL SHOOT ASAP"

We will do photos when we're ready. Right now we're just focused on getting through the first weeks.



Chapter 7: Body and Emotional Commentary

The third comment of the visit is almost always about your body.

"You look tired." "Have you slept?" "Wow, you bounced back fast." "You look so [adjective]." Each is meant kindly. Each is also a comment on your appearance from a person who has not been asked to comment on your appearance. The cumulative effect, over a postpartum stretch full of these, is exhausting.

You do not have to play along. You do not have to perform "fine" for the visitor. You do not have to laugh off the body comment. "Yes. Postpartum." is a complete sentence. So is silence.

The three scripts in this chapter are short. They are short on purpose. The right response to "you should be enjoying every minute" is not a long one. It is "some minutes are better than others," and then a topic change.

A reminder: people commenting on your body are not entitled to your reaction. You owe them no laugh, no reassurance, no "I'm fine!" performance. You can be honest, brief, and warm at the same time.

SCRIPT 6.1, THE "YOU LOOK TIRED" / "HAVE YOU SLEPT?" PERSON

Yes. Postpartum.

(That's the whole script. Use as needed.)

SCRIPT 6.2, THE "ARE YOU SURE YOU'RE OKAY?" WITH A WORRIED FACE

I'm doing what I need to do. If something changes I'll let you know.

SCRIPT 6.3, THE "YOU SHOULD BE ENJOYING EVERY MINUTE" PERSON

Smile. Pause. "Some minutes are better than others." Then change the topic.

Chapter 8: Templates for the Bigger Conversations

Sometimes a single script does not do the work.

For the family group text where you set the expectations before the baby arrives. For the email you send announcing the visitor policy. For the "I changed my mind" follow-up when a yes turned out to be a no. For the harder one-on-one with a family member whose behavior is making your recovery worse, not better.

These are templates, not scripts. You will modify them. The bones are here.

A note before you send any of these: read them once with your partner if you have one. They will catch the sentence that lands too sharp. They will also remind you when you are softening something that needs to be firm.

TEMPLATE 1, THE PRE-BABY FAMILY TEXT

Subject: Heads up for baby's arrival

Hi everyone,

As [we / I] get closer to baby's arrival, [we / I] wanted to share how [we're / I'm] handling the first few weeks. Please don't take any of this personally, these are decisions [we've / I've] made for [our / my] mental and physical health.

1. No hospital visitors. We will share photos as soon as we can.
2. No visitors at our home for the first [2 / 3 / 4] weeks. We will reach out individually when we're ready for visits.
3. Please don't drop by unannounced, even if it's "just a quick stop." We will not be answering the door.
4. If you'd like to help in those first weeks, the most useful things are: meals on the porch (no need to come in), gift cards for food delivery, or just a "thinking of you" text. Please don't take it personally if we don't respond right away.
5. Please don't share photos of [baby] on social media. We'll let you know when (or if) that's okay with us.

Thank you for understanding. We love you all and we're excited for [baby] to meet you when we're ready.

[your name]

TEMPLATE 2, THE "I CHANGED MY MIND" FOLLOW-UP

Hi [name],

I know I said [X], and I want to be honest, that's not working for us anymore. We need to [change to Y].

I know this might be disappointing. I'm not in a place to over-explain it right now. I hope you can understand.

[your name]

TEMPLATE 3, THE "WE NEED SPACE" HARD CONVERSATION

[Family member name],

I love you. I need to be honest about something that's been hard for me.

When you [specific thing, drop by unannounced / comment on the baby's weight / give me parenting advice / etc.], it's making my recovery harder, not easier.

I'm not asking you to stop loving us or stop wanting to be involved. I'm asking you to [specific request: please call before coming / please keep parenting advice to yourself unless I ask / please drop food on the porch instead of coming in].

I'd like for us to be close again. Right now I need this from you.

[your name]

TEMPLATE 4, THE "NO PRESENTS AT THE SHOWER" BOUNDARY

We are so excited to celebrate with you. As a small ask, please no gifts. We have everything we need, and our space is small.

If you really want to give us something, we'd love a frozen meal, a gift card for [grocery / restaurant delivery], or simply your company.

Thank you for understanding!

TEMPLATE 5, THE "THANKS BUT NO" GIFT SCRIPT (FOR UNWANTED HAND-ME-DOWNS, RELIGIOUS ITEMS, ETC.)

[Name], thank you so much for thinking of [baby]. This is really generous of you.

We've decided that [reason, we're not using X / we don't have space for Y / we're going a different direction with Z]. I hope you don't take this personally.

If it's okay with you, we'll [pass it along / donate it / return it].

Thanks for understanding!

Chapter 9: How to Use These Scripts

The scripts are a starting place, not a finish line. A few principles for using them well.

Save them somewhere reachable. The reason scripts work is that they are pre-loaded. You are not finding the words in the moment. You are pulling them up. Save the relevant ones to your notes app. Send a copy to your partner. Pin the door-sign template to your fridge before the baby is born.

Modify freely. Replace the names. Swap the warmth dial. Add or remove the heart emoji. The scripts are calibrated for warm-direct, but your voice is yours. A script that sounds like someone else is harder to send than one that sounds like you.

Share with your partner. They need scripts too. Different ones. The "for your partner at the door" script is the one to start with. If your partner is the person speaking to their own family, they need to be the one delivering the lines. You delivering the lines for them creates a different dynamic.

Do not justify. "No" is a sentence. Adding reasons gives the other person something to argue with. "We are not having visitors right now" is final. "We are not having visitors right now because the pediatrician said RSV season is bad and my mother already came over last week and we want some space" is six openings for negotiation.

Repeat as needed. Boundaries are not a one-time conversation. Some people need to hear the boundary three times. The third repeat is allowed to be shorter and firmer than the first. "We are

not seeing visitors right now" can be said multiple times, with progressively less softening.

Forgive yourself for changing your mind. Saying yes one week and no the next is fine. Your needs change. Postpartum hormones change. The baby's needs change. The family member's behavior changes. You are allowed to update.

Remember the JADE rule. Do not Justify, Argue, Defend, or Explain. Each of those is an invitation to a longer conversation. Pick a warm-firm sentence. End there.

Do not respond to texts in real time. The cost of postpartum is your reactive bandwidth. Looking at a text and responding in five minutes is not required. Two hours later is fine. Tomorrow morning is fine. The world will not collapse because you did not respond to a "can I come over" text within the hour.

Chapter 10: A Last Note

Some of the people you set boundaries with are going to be hurt.

Some are going to be angry. Some are going to tell other people you are being "selfish" or "hormonal" or "not yourself right now." A few might withdraw entirely for a stretch and re-emerge later when they have had time to adjust. A small number will not re-emerge.

That is the cost of saying no, and the cost is the price of the season.

You are yourself. You are *exactly* yourself. You are just yourself with new information about what you need.

The right people will respect you for this. The people who love you with kindness, not pressure, are going to hear "we are not having visitors right now" and say "okay, we will be here when you are ready." They are not going to need to be convinced. They are going to make it easy.

The wrong people will reveal themselves. The ones who keep pushing, who guilt, who escalate, who say the thing about how the baby is also their family member, who post the photo after you asked them not to, those people are showing you something useful. They are showing you what your relationship with them is going to look like in the next ten years.

You are not creating that information. You are just being given it.

Either outcome is useful.

Take what you need from this book. Use the scripts that fit your voice. Ignore the ones that do not. Send the relevant chapters to

your partner. Print the door sign. Tape it to the fridge. Save the templates to your notes app.

The work of saying no is unglamorous. It is also the work that protects the season you are in. The baby is little for a short stretch. Your recovery deserves the quiet. The boundaries you hold now are the foundation for the family you are building.

You are doing the work.

With care, Maya

About Maya

Maya is the voice behind Soothemade. A postpartum doula and mother of two, she writes the books and printables she wishes she had when she was on the third day home from the hospital, holding cookies she did not ask for, smiling at a relative she could not get to leave.

She is a fictional founder, a composite of every parent who has emailed Soothemade to say "I felt like you had been in my house." That composite voice is the brand voice. The decisions, the products, and this book were all written under that voice.

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents.

The books exist for the moments when a journal does not fit the need. When you want to read, not write. When you are looking up the chapter at 3am on your phone, not pulling a printed page out of a drawer.

Each book is adapted from a Soothemade printable. The printable version contains writing space and prompts. The book version contains the reading. Both work. Some readers want one, some want the other, some want both. You can find the printable version of every book at notes.soothemade.com.

Also from Soothemade

The printable companion to this book is **The "Saying No" Script Pack**, available at notes.soothemade.com/shop/saying-no-scripts. The scripts and templates are the same. The printable is sized to print, fold, and tape to the fridge.

Other Soothemade products that pair with this book:

- **The Partner's Postpartum Playbook**, for the non-birthing partner. Many of the scripts in this book are most effective when your partner is the one delivering them. The Playbook is their companion.
- **The Visitor Welcome Packet**, for when you are ready for visitors. Ground rules, food drop menu, "how to help" list, and a meal-train tracker.
- **The Asking for Help Script Pack**, the inverse of this book. Thirty scripts for asking specific people for specific help. The book version is the second Soothemade title, coming next.

Crisis lines

If thoughts of harming yourself or your baby are present, call.

United States

- 988, Suicide and Crisis Lifeline (call or text)
- 1-833-852-6262, Postpartum Support International HelpLine (call or text)
- 1-833-943-5746, Maternal Mental Health Hotline (24/7)

United Kingdom

- 116 123, Samaritans

Canada

- 9-8-8, Suicide Crisis Helpline

Australia

- 1300 726 306, PANDA, Perinatal Anxiety and Depression Australia

Postpartum mood disorders are common and treatable. Asking for help is not a sign that something is wrong with you. It is a sign that you are paying attention.

This book is for planning and personal use only. It is not legal, medical, or psychological advice. For specific concerns about your postpartum recovery or your baby's health, please follow your obstetric and pediatric providers' guidance.

A Soothemade book. Made slowly, in plain language.